

PERCEPTION OF FEMALE GENITAL MUTILATION AMONG SECONDARY SCHOOL STUDENTS IN ENUGU EDUCATION ZONE

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Abstract

The study explored perception of female genital mutilation among secondary school students in Enugu Education Zone. Descriptive survey design format was adopted and the population for the study comprised of 14312 (4525 males, 9787 females) SS1 and SS2 students from the 33 public secondary schools in Enugu Education Zone A total of 389 (123 males and 266 females) determined through Taro Yamane formula from 9 out of 33 public secondary schools served as research participants. The analysis of data was carried out with mean and standard deviation on while the hypothesis were tested with t-test statistic at .05 level of significance. Findings revealed that secondary school students have positive perceptions of FGM in Enugu Education Zone ($\bar{x}=2.66$). Female secondary school students have higher perceptions of short-term health consequences of FGM than their male counterparts in Enugu Education Zone ($m, \bar{x}=2.45$; $F, \bar{x}=2.62$). However, both the male and female secondary school students have negative perceptions of long-term health consequences of FGM in Enugu Education Zone ($m, \bar{x}=2.37$; $F, \bar{x}=2.48$). There is no significant difference in the mean responses of male and female students regarding perceptions of FGM and its short-term health consequences ($P>.05$). However, significant difference exist in students perceptions of long-term health consequences of FGM (PFGM in the favour of the female students. The recommendations among others include interventions in form of advocacy in order to create more awareness on both short-term and long-term consequences associated with the practice of FGM.

Key words: Health, female Gender Mutilation, perception, Student

Introduction

Genital cutting is one of the controversial reproductive practices undertaken in males and female gender in the course of transiting from infant to adulthood. In females, it is generally regarded as female genital mutilation (FGM). According to the World Health

Organization (WHO), (2018), female genital mutilation encompasses procedures involving the partial or total removal of external genitalia or other injury to female genital organs for non-medical reasons. That is to say that the practice of female genital cutting is non-medical procedure involving the removal, partial or complete of the external female genitalia or other injuries to the female genital organs. Unlike the male mutilation that is generally acceptable in greater part of the world, the female genital mutilation (FGM) is not approved in most societies even with the high prevalence ratio worldwide.

Globally, it has been noted that over 200 million girls and women have undergone FGM, leading to severe physical, physiological and psychological consequences such as menstrual problems, painful urination, anxiety and others. Specifically, approximately 130 million women have been subjected to FGM, with

an additional 2 million new cases annually in regions such as Africa, the Middle East, and Southeast Asia (World Health Organization, 2020). Similarly, the United Nations International Children Education Fund (UNICEF), estimates that 3 million African girls are at risk of FGM each year and warns that population growth could increase the overall number of affected individuals in the next 15 years without effective prevention efforts (UNICEF, 2019). Female genital cutting (FGC) also known as FGM is one of the harmful cultural practices that must be combated and eradicated in all countries, not just in Africa but also among all nations of the world.

FGC is a practice that is practiced in at least 28 African countries, as well as certain Middle Eastern and Asian countries. In addition, immigrant groups in Western countries, such as Europe, Canada, Australia, New Zealand, and the United States, have also been observed engaging in

similar behaviour in their new home. About 140 million women and girls are exposed to FGC, with an additional 2 million being subjected to the procedure each year (United Nations 2018; Gomes 2018). The statistics provides strong evidence that FGM practices are evidenced in many civilizations including Nigeria.

In Nigeria, the practice varies by region, with higher prevalence in the South-West and South-East, primarily among the Yoruba and Ibo tribes. The prevalence in other regions is as follows: South-South (34.2%), North-West (19.6%), North-Central (11.4%), and North-East (2.7%) (Odukogbe, Afolabi, Bello & Adeyanju, 2017). In Nigeria, FGM is practiced in various forms, with Types I, II, and III being the most common, and Type IV is practiced differently in the North and South regions. It is performed on girls at different stages of life, including infancy, childhood, adolescence, before marriage, during the

first pregnancy, or even at death. The practice is found across social classes and religious groups, including Muslims, Christians, and traditional worshippers and most often performed on girls between the ages of 4 and 12 to mark their transition to womanhood. With about 200 million people, Nigeria has the highest absolute number of cases of FGM in the world, accounting for about one-quarter of the estimated 115–130 million circumcised women worldwide (Siddhanta & Sinha, 2016). From the available statistics, the prevalence ratio with respect to Nigeria situation which classifies the southeast geopolitical area high in terms of the practice of it is worrisome. However what has not been successfully established is whether the perception is positive or negative particularly among secondary school students. Hence, it becomes crucial that a study focusing on perception of FGM among secondary school students in Enugu

State and specifically in Enugu Education Zone.

Despite efforts to discourage the practice, it remains widespread in Nigeria, with over 50% of Nigerian women estimated to have undergone it (Odukogbe et al. 2017). The practice's widespread popularity is based on various religious and socio-cultural factors. The urge to comply with societal conventions, gender identity, marital acceptability, social acceptance, virginity preservation, suppression of sexual desires, deep cultural tendencies, and religious instructions are only a few (Daniyan, Sunday-Adeoye, Ekwedigwe. & Dantani, 2018).

There are other adduced reasons for embarking on FGM in Nigeria. Results from Nigerian studies found that some of the reasons for keeping the practice in place were to keep girls pure as virgins, protect the girls' reproductive potential, increase fertility, and aid the childbirth process.

There are beliefs among practicing communities that uncut women have higher rates of stillbirths because if the baby's head touches an uncut clitoris, death will occur. However, controlling the sexuality of women and reducing their desire for sex to protect them were reasons for sustaining the practice in several other cultures. For married women, a reason for the continued practice of FGC is that it limits the desire for sex due to pain during sexual intercourse and reduces the tendency for extramarital sex. Furthermore, FGC is believed to make the woman more attractive physically and socially and make the genitalia more attractive, ensure girls' health, pubertal maturation, and genital cleanliness (Kallon & Dundes, 2013; FMOH, 2013; Omigbodun et al., 2020). Despite the adduced reasons, there exist contrary opinions. The argument has been that Female genital mutilation/cutting violates women's and girls' rights since it can cause long-term

physical and emotional trauma and many long-term and short-term issues (Adinew & Mekete 2017).

FGM is a problem that has taken several dimensions and continues to be practiced despite enormous efforts and resources invested by governments, non-governmental organizations (NGOs) and other stakeholders to ensure that the practice is discontinued worldwide. However, in the southeast Zone of which Enugu Education Zone is part and parcel of, the perception of its practice needs to be ascertained in view of the health consequences associated with it. On the other hand, FGC has yet to demonstrate any known health benefits. However, scholars such as Daniyar et al. (2018), claim that it has major medical and psychosocial consequences. Notable health consequences of FGM indentified in literature includes severe haemorrhage, shock, trauma, pain, sepsis, death, and long-term issues such as chronic pelvic pain,

infertility, painful intercourse, reduced sexual desire, marital disharmony, emotional trauma, coital bleeding, increased need for an episiotomy, obstetric haemorrhage, and obstetric fistula (Yasin et al, 2018). It has been demonstrated that circumcised women have been reported to have higher complications in labour. Yet, despite all the adverse effects of female genital circumcision, the practice has continued, owing to its strong roots in cultural ideas (Odukogbe et al. 2017; Epundu., 2018). From the reviewed reports, perceptions of FGC varies with diverse groups including perceptions of short-term health consequences of its practice.

Short-term health consequences of FGM have been documented in literature. Notable short-term immediate physical health consequences may include injury to adjacent tissue of the urethra, vagina, perineum and rectum, shock, severe pain, heavy bleeding, acute urinary retention,

pelvic inflammatory disease, risk of contracting infections such as hepatitis B, difficulty in passing urine, loss of normal sexual function, cysts and abscess on genital, repeated pain during intercourse and menstruation, among others (Adinew & Mekete 2017). Secondary school students' perceptions of short-term health consequences of FGM if identified could be a guide for interventions not only on short-term basis but also on long-term health consequences of it.

Long-term health consequences may ensue following failure to identify and address the short-term consequences. Long-term complications include urine retention and associated urinary tract diseases, infections, obstruction in menstrual cycles and other similar reproductive-tract infections, infertility, painful intercourse, psychological and sexual problems, and prolonged and obstructed labour (WHO, 2016). Also, the risk of transmission of the

human immunodeficiency virus (HIV) is considered a factor in FGC. Furthermore, when girls are cut in group ceremonies, circumcisers use the same instrument on all the initiates. This practice tends to spread infectious diseases such as HIV. Furthermore, even after it has undergone a remedial process, an infibulated woman's damaged or shrivelled vulva has a high possibility of being effortlessly torn during sexual intercourse, with an attendant vulnerability of HIV transmission by an infected partner (Adinew & Mekete 2017). These identified short and long term health consequences of FGM forms the rational for carrying out this investigation in order to ascertain secondary school students' perceptions of them in Enugu Education Zone.

Perceptions of short-term and long-term health consequences of FGM may be positive or negative depending on the individual concerned. In like manner,

secondary school students' perceptions of FGM may be positive or negative. According to Nworah and Ebere (2015), perception refers to the general feeling and idea upon which people observe and see things within their environment. The authors point out that perceptions are mental frames of reference that could be based on either founded or unfounded statements, a real or imagined state of affairs. This also means that secondary school students' generate feelings and ideas about FGM which tend to determine how far it influences their health which may be positive or negative.

Studies on perceptions of FGM have been documented in literature. It has been observed that FGM is considered essential for women's socialization, curbing their sexual appetites and preparing them for marriage (NPC Nigeria, 2014). In a study of circumcised women's attitudes towards female circumcision in a Nigerian community where the practice is accepted

showed that 62 percent of the 100 interview subjects from all social strata favoured the practice as an instrument for controlling female sexuality and cultural pride (Briggs, 2018). According to Bodunrin (2019), FGC is identified simply as a cultural obligation and cleansing rite, with people describing it as female circumcision, believing it is equivalent to male circumcision. Mockery, loss of respect, and reduced marriage offers are social sanctions against non-circumcised females in Nigerian cultures where FGC is practiced (Briggs 2018). According to Omigbodun et al. (2020), understanding FMC's socio-cultural determinants is key to identifying the factors that continue to drive it. Anuforo, Oyedele and Pacquiao (2014), suggest that FGC is viewed as a non-negotiable rite of passage into womanhood that improves the social value of a woman in every society where it is practiced. There is great superiority placed on the ability of women to marry and in the belief that FGC

ensures purity and protection for the girl-woman before marriage (Inhorn & Buss, 2013). Furthermore, Mackie (2013), observes that the importance of a daughter's marriageability far outweighed the importance of health, consent, and maintenance of body integrity. Boddy (2014), describes FGC as a practice that protects and prepares the woman for motherhood and ensures fertility.

This line of argument stems from one of the theories of purity, the social theory which holds the assumption that attitudes are determined by one's perceptions of the consequences of his or her actions. One example of normative considerations is social pressure to perform or not perform a specific behaviour hence consciousness and purpose affect human behaviour. What this implies is that the impressions of people in the community may influence female secondary school students' willingness to obtain genital mutilation if they have not

done so but could have better perceptions with the realization that undertaking such act could result in negative health condition. However, such dispositions could exhibit differentials by gender as male students' perceptions of FGM may differ from their female counterparts in Enugu Education Zone.

Gender could shape health behaviour and may determine students' perceptions of FGM. In that regard, it could be possible for the male secondary school students to be keeping positive attitude towards FGM with their female counterparts probably keeping negative dispositions but which has not been assessed empirically hence the present study. With improvement in education and social status of women and increased awareness of complications of FGC, most women who underwent FGC disapprove of the practice, and only very few are prepared to subject their daughters to such harmful procedures (Mandara 2014). This perceptions may not

be applicable with men hence the focus of this study on male and female students in secondary schools who will eventually become future mothers and fathers. Hence this study explored secondary school students' perceptions of FGM in Enugu Education Zone.

Statement of the Problem

Female genital mutilation is a deeply rooted cultural practice undertaken in all communities in Enugu State. This also implies that females in the state are expected to have positive perceptions of it right from home. However, because genital mutilation in Ibo land comes at a child's early stages of life also makes it impossible for her to make a decision whether to obtain it or not and as such her perceptions of it may be negative. The negative perceptions may be due to the health consequences associated with genital mutilation such as injury to adjacent tissue of the urethra, vagina, perineum and rectum, shock, severe pain, heavy bleeding, acute

urinary retention, pelvic inflammatory disease, risk of contracting infections such as hepatitis B, difficulty in passing urine, loss of normal sexual function, cysts and abscess on genital, repeated pain during intercourse and menstruation. Other long-term effects include urine retention and associated urinary tract diseases, infections, obstruction in menstrual cycles and other similar reproductive-tract infections, infertility, painful intercourse, psychological and sexual problems. Consequently, the female girl in secondary school may begin to develop negative perceptions of the act. Furthermore, if the girl child in secondary school obtained genital mutilation in the time past, she may begin to develop some psychological trauma. Findings on the perception of girls in secondary schools could be a stepping stone for interventions..

Purpose of the Study

The main purpose of this study is ascertain the perception of FGM among

female secondary school students in Enugu Education Zone. The specific purposes were to;

1. find out the perception of FGM among secondary school students in Enugu Education Zone.
2. ascertain the perception of short-term health consequences of FGM among secondary school students in Enugu Education Zone based on gender.
3. investigate the perception of long-term health consequences of FGM among secondary school students in Enugu Education Zone based on gender.

Research Questions: The following research questions were answered in the course of this study.

1. What is the perception of FGM among female secondary school students in Enugu Education Zone?
2. What is the perception of short-term health consequences of FGM among secondary school students in Enugu Education Zone based on gender?
3. What is the perception of long-term health consequences of FGM among female secondary school students in Enugu Education Zone based on gender?

Hypotheses: The following null hypotheses were tested at .05 level of significance.

1. There is no significance difference in the mean responses of male and female secondary school students on perception of FGM in Enugu Education Zone.
2. There is no significance difference in the mean responses of male and female secondary school students on

perception of short-term FGM in
Enugu Education Zone

3. There is no significance difference in
the mean responses of male and
female secondary school students on
perception of long-term FGM in
Enugu Education Zone

Method: On research method adopted for this study, descriptive survey format was adopted and the population for the study comprised of 14312 (4525 males, 9787 females) male and female SS1 and SS2 students from the 33 public secondary schools in Enugu Education Zone according to Post Primary Schools Management Board (PPSMB) , Enugu, Enugu State, 2024/2025. A total of 389 (123 males and 266 females) male and female secondary school students determined through Taro Yamane formula from 9 out of 33 public secondary schools served as research participants. On sampling techniques multi-stage procedure involving three stages namely stratified sampling,

purposive and systematic sampling techniques were deployed in selecting the respondents. Under this procedure, Enugu North and Enugu East were sampled as urban while Isi-Uzo was rural. The instrument for data collection was researcher's made, structured and validated questionnaire titled "Questionnaire on Perception of FGM (QPFGM)". The instrument QPFGM was validated by three experts while the internal consistency was determined cluster by cluster with the overall reliability index of .77 which shows it is reliable and suitable for the study. The instrument was personally administered by the researcher during school hours with the help of three research assistants who are form teachers in the sampled classes of the sampled schools and retrieved on the spot. After the administration, the researcher discovered that seven of the respondents did not return theirs which left the number used for data analysis at 382.

The analysis of data was carried out with mean and standard deviation while the hypotheses formulated were tested with t-test statistic at .05 level of significance at appropriate degree of freedom. The decision rule was based on real limit of numbers as follows; Mean values from 3.50– and above – Positive perception. Mean values from 2.50 – 3.49 – Positive perception. Mean values from 1.50 – 2.49 – Negative perception. Mean values from 0.00 – 1.49 – very negative perception (Mogul, 2017).

Furthermore, high standard deviation values ($x > 2.0$) (that is mean values obtained greater than) indicated

existence of extreme scores (hence, unreliable mean). Lower standard deviation values ($x < 2.0$) indicated non-existence of extreme scores (hence, a reliable mean). This was also based on the recommendations of Mogul (2017). t-test was used to test the hypotheses at 0.05 significant level. When the computer generated significant levels are equal to or greater than 0.05 ($p \geq 0.05$), the t-test was considered not significant. Hence, hypotheses were not rejected. Otherwise ($p < 0.05$) the t-test was considered significant, thus, hypotheses was not rejected, (Krecks 2019).

Results: The results are presented in Tables.

Research Question 1: What is the perception of FGM among secondary school students in Enugu Education Zone?

Table 1: Mean and SD Ratings of Male and Female Respondents on Perception of FGM

S/ N	Indicate your perceptions of FGM;	Male (121)								Female (261)						Overall (382)		
		SA	A	D	SD	X	SD	Dec	S A	A	D	SD	X	SD	Dec	X	SD	Dec
1	FGM is equivalent to male cutting	38	36	29	18	2.78	.22	+ve	88	73	69	37	2.86	.23	+ve	2.382	.23	+ve
2	It is suppose to prepare a girl for motherhood.	32	34	30	25	2.60	.28	+ve	59	55	77	70	2.39	.3	-ve	2.49	.29	-ve
3	FGM is a violation of the fundamental human rights of girls.	32	33	27	29	2.56	.47	+ve	89	83	58	31	2.88	.46	+ve	2.72	.46	+ve
4	It ensures purity and protection for the girl before marriage.	39	41	22	19	2.83	.49	+ve	77	79	68	37	2.75	.47	+ve	2.79	.48	+ve
5	FGM is a cultural obligation and cleansing rite.	40	38	19	24	2.78	.40	+ve	63	55	76	67	2.44	.41	-ve	2.2.61	.4	+ve
6	FGM lacks health benefits	27	25	39	30	2.40	.38	-ve	83	88	60	30	2.86	.38	+ve	2.63	.38	+ve
7	It should be abolished in my community.	30	26	33	34	2.46	.38	-ve	88	74	68	37	2.86	.37	+ve	2.66	.38	+ve
8	Women who are mutilated are faithful to their husbands.	42	44	15	20	2.89	.33	+ve	70	74	69	48	2.64	.57	+ve	2.77	.38	+ve
9	It causes physical and psychological trauma to the girl	34	33	30	26	2.65	.37	+ve	68	78	50	65	2.57	.40	+ve	2.61	.39	+ve
	Grand Mean					2.66	.37	+ve					2.69	.78	+ve	2.66	.56	+ve

Key: +ve-Positive Perception. -ve-Negative Perception.

Table 1 data that deals with perception of FGM among secondary school students in Enugu Education Zone. The Table shows that the male respondents have positive perceptions of items 1, 2, 3, 4, 5, 8 and 9 while items 6 and 7 recorded negative perceptions. The female students on the other hand had positive perceptions of items 1, 3, 4, 6, 7, 8 and 9 while items 2 and 5 recorded negative perceptions. Furthermore, the male students had a total mean score of 2.66 with SD .37 while the female is 2.69 with SD .78. This signifies positive perception. The grand mean of 2.66 with SD .57 signifies positive perception. This therefore, means that secondary school students have positive perceptions of FGM in Enugu Education Zone.

Research Question 2: What is the perception of short-term health consequences of FGM among secondary school students in Enugu Education Zone?

Table 2: Mean and SD Ratings of Male and Female Respondents on Perception of Short-Term Health Consequences FGM

S/ N	Indicate your perception of short-term consequences of FGM;	Male (121)							Female (261)							Overall (382)		
		SA	A	D	SD	X	SD	Dec	SA	A	D	SD	X	SD	Dec	X	SD	Dec
10	It gives injury to the urethra	25	30	34	32	2.23	.21	-ve	86	75	37	69	2.40	.25	-ve	2.32	.23	+ve
11	Injury to the vagina comes with FGM	40	34	18	29	2.24	.24	-ve	78	88	33	62	2.70	.33	+ve	2.47	.29	-ve
12	Heavy bleeding comes with FGM	45	41	19	16	2.80	.42	+ve	83	87	57	34	2.84	.43	+ve	2.82	.43	+ve
13	Acute urinary infection usually occurs following mutilation	19	16	45	41	2.43	.41	-ve	77	79	55	50	2.70	.41	+ve	2.57	.41	+ve
14	Pelvic inflammation occurs after mutilation	17	22	38	44	2.35	.35	-ve	79	81	61	40	2.76	.35	+ve	2.56	.35	+ve
15	There is the risk of contracting infections such as hepatitis B	29	25	33	34	2.26	.33	-ve	80	83	66	32	2.81	.37	+ve	2.54	.35	+ve
16	There is total loss of sexual functions	15	21	37	53	2.41	.33	-ve	40	60	82	79	2.23	.43	-ve	2.32	.38	-ve
17	It causes painful menstruation	39	42	31	9	2.45	.36	-ve	66	57	70	68	2.46	.40	-ve	2.46	.30	-ve
	Grand Mean					2.45	.33	-ve					2.61	.37	+ve	2.53	.35	+ve

Key: +ve-Positive Perception. -ve-Negative perception.

Table 2 data that deals with perception of short-term health consequences of FGM among secondary school students in Enugu Education Zone. The Table shows that the male respondents have positive perceptions of only items 12 while items 10, 11, 13, 14, 15, 16 and 17 all recorded negative perceptions. The female students on the other hand had positive perceptions of items 11, 12, 13, 14 and 15 while items 10, 16 and 17 recorded negative perceptions. Furthermore, the male students had a total mean score of 2.45 with SD .33 while the female is 2.61 with SD .37. This signifies positive perception. The grand mean of 2.53 with SD .35 signifies positive perception. This therefore, means that female secondary school students have higher perceptions of short-term health consequences FGM than their male counterparts in Enugu Education Zone.

Research Question 3: What is the perception of long-term health consequences of FGM among secondary school students in Enugu Education Zone?

Table 3: Mean and SD Ratings of Male and Female Respondents on Perception of Long-Term Health Consequences of FGM

S/ N	Indicate your perception of long-term consequences of FGM;	Male (121)							Female (261)							Overall (382)		
		SA	A	D	SD	X	SD	Dec	SA	A	D	SD	X	SD	Dec	X	SD	Dec
18	FGM causes urine retention.	23	27	39	32	2.34	.32	-ve	77	71	67	46	2.69	.24	+ve	2.52	.28	+ve
19	It is associated with associated urinary tract diseases in girls.	25	27	35	34	2.36	.38	-ve	70	75	59	57	2.61	.36	+ve	2.49	.37	-ve
20	FGM causes urine retention.	21	19	44	37	2.20	.44	-ve	46	67	77	71	2.34	.26	-ve	2.27	.35	-ve
21	FGM causes obstruction in menstrual cycles	23	26	37	35	2.31	.45	-ve	66	47	70	78	2.39	.37	-ve	2.35	.41	-ve
22	It causes infertility in girls	18	24	39	40	2.17	.41	-ve	59	57	70	75	2.38	.31	-ve	2.28	.36	-ve
23	It causes reproductive tract infections.	26	21	40	34	2.32	.36	-ve	61	56	69	75	2.39	.34	-ve	2.36	.35	-ve
24	FGM causes painful intercourse	30	22	38	31	2.42	.32	-ve	59	66	69	67	2.45	.27	-ve	2.44	.30	-ve
25	It brings about psychological problems	37	38	33	13	2.82	.33	+ve	71	69	63	58	2.59	.37	+ve	2.71	.35	+ve
Grand Mean						2.37	.38	-ve					2.48	.32	-ve	2.43	.33	-ve

Key: +ve-Positive Perception. -ve-Negative perception.

Table 3 data that deals with perception of long-term health consequences of FGM among secondary school students in Enugu Education Zone. The Table shows that the male respondents have positive perceptions of only items 25 while items 18, 19, 20, 21, 22, 23 and 24 all recorded negative perceptions. The female students on the other hand had positive perceptions of items 18, 19 and 25 while items 20, 21, 22, 23 and 24 recorded negative perceptions. Furthermore, the male students had a total mean score of 2.37 with SD .38 while the female is 2.48 with SD .32. This signifies negative perception. The grand mean of 2.43 with SD .33 signifies negative perception. This therefore, means that both the male and female secondary school students have negative perceptions of long-term health consequences FGM in Enugu Education Zone.

H0₁: There is no significance difference in the mean responses of male and female secondary school students on perception of FGM in Enugu Education Zone.

Table 2: Summary of t-test analysis on the mean responses of male and female female secondary school students on perception of FGM in Enugu Education Zone.

Group	N	$\bar{x}$	SD	Df	Level of Sig	P-value	Decision
Male	121	2.66	.37	380	.05	.093	Ho ₁ not rejected
Female	261	2.69	.78				

Data in Table 2 for male and female secondary school students on perception of FGM show that at 380 degree of freedom, the p-value was .093. The outcome of the p-value is greater than .05 level of significance set for this study. This implies that the null hypothesis was not rejected and, therefore, there was no significant difference between the mean ratings of male and female secondary school students on perception of FGM in Enugu Education Zone.

H0₂: Significant difference does not exist between the mean responses of male and female secondary school students on perception of short-term health consequences FGM in Enugu Education Zone

Table 4: Summary of t-test analysis on the mean responses of male and female secondary school students on perception of short-term health consequences FGM

Group	N	$\bar{x}$	SD	Df	Level of Sig	P-value	Decision
Male	121	2.45	..33	380	.05	.031	Ho ₂ rejected
Female	261	2.61	.37				

Data in Table 4 for male and female secondary school students on perception of short-term health consequences FGM show that at 380 degree of freedom, the p-value was .031. The outcome of the p-value is less than .05 level of significance set for this study. This signifies that the null hypothesis was rejected and, therefore, there is a difference between the mean ratings of male and female secondary school students on perception of short-term health consequences FGM in Enugu Education Zone.

Ho₃: Significant difference does not exist between the mean responses of male and female secondary school students on perception of long-term health consequences FGM in Enugu Education Zone

Table 6: Summary of t-test analysis on the mean responses of male and female secondary school students on perception of long-term health consequences FGM

Group	N	$\bar{x}$	SD	Df	Level of Sig	P-value	Decision
Male	121	2.37	..38	380	.05	.061	Ho ₃ rejected
Female	261	2.48	.32				

Data in Table 6 for male and female secondary school students on perception of long-term health consequences FGM show that at 380 degree of freedom, the p-value was .061. The outcome of the p-value is less than .05 level of significance set for this study. This signifies that the null hypothesis was not rejected and, therefore, there is no significant difference between the mean ratings of male

and female secondary school students on perception of long-term health consequences FGM in Enugu Education Zone.

Discussion

The finding of the study revealed that secondary school students in Enugu Education Zone possess positive perceptions of FGM. This finding agrees with the work of Briggs (2018), which found 62% of circumcised women's attitudes and perceptions towards female circumcision in a Nigerian community where the practice is accepted positive as they generally agreed that FGM is an instrument for controlling female sexuality and cultural pride. The finding also support the work of Bodunrin (2019), which identified FGM as a cultural obligation and cleansing rite, with people describing it as female circumcision, believing it is equivalent to male circumcision. The finding also fall in line with Mackie (2013), which demonstrated that the importance of a daughter's marriageability far outweighed the importance of health, consent, and maintenance of body integrity while Boddy (2014), describes FGC as a practice that protects and prepares the woman for motherhood and ensures fertility.

The agreement with previous report could be because some of the respondents are rural dwellers where the practice of FGM is part and parcel of their culture and may view it as norm. Perhaps, the positive perceptions of FGM may have been acquired through the social media.

Female secondary school students have higher perceptions of short-term health consequences FGM than their male counterparts in Enugu Education Zone. This finding agrees with Odukogbe et al. (2017) and Epund.(2018), which demonstrated that circumcised women have been reported to have higher complications in labour but notwithstanding the the practice has continued, owing to its strong roots in cultural ideas. It could be possible that some the female respondents may have witnessed discussions by their parents about possible dangers associated with the practice of FGM. Perhaps, some of their friends who probably got married on time may have experienced one or more of the health conditions associated FGM unlike their male

counterparts who may not take much interest.

Another finding of this study shows that both the male and female secondary school students possess negative perceptions of long-term health consequences of FGM in Enugu Education Zone. The finding disagree with Anuforo, Oyedele and Pacquiao (2014), which viewed FGC as a non-negotiable rite of passage into womanhood that improves the social value of a woman in every society where it is practiced due to great superiority placed on the ability of women to marry and in the belief that FGC ensures purity and protection for the girl-woman before marriage. It could be possible that majority of the respondents never witnessed the practice growing up or probably never had it. Hence. They not be familiar with the likely health condition that may arise following the act.

Conclusion: From the findings of this study, it was concluded that there are variations in male and female students in secondary schools perceptions of FGM in the favour of the female students.

Recommendations: The study recommend as follows:

1. The positive perceptions of FGM by secondary school students in the Zone requires interventions in form of advocacy in order to create more awareness on both short-term and long-term consequences associated with the practice of it.
2. The higher perceptions of short-term health consequences of FGM by the female students than their male counterparts means that the school authority, Health Educators and counselors should target the males in order to equip them with knowledge and skills to resist the practice during family life and living.
3. Both the male and female students should be sensitized through seminars and symposium on both the short and long-term health consequences of FGM for possible import to the general population.

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